SAMPLE ANSWER · SPINAL CORD INJURY · CASE 3

Sample 7-minute ISBAR: Frank

Frank Doyle · 77-year-old man · Acute orthopaedic and spinal ward, public hospital · about 1044 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

55 words

"Hi, I'm the occupational therapy student who has been completing Frank Doyle's pre-discharge breakfast and self-care assessment. Frank is 77, with an incomplete C4 AIS D spinal cord injury with a central cord pattern from a fall three weeks ago. I have two concerns: one needs action right now, and one affects tomorrow's planned discharge."

S Situation

141 words

"The immediate concern is his cervical collar. It is documented that it must be worn at all times, but when I arrived at 8 am he was shaving with the collar off on the table. He says he removes it every morning and after his shower to wash and shave because it itches, and he hasn't told the nurses. He is now trying to put it back on himself.

"The second concern is his hand function. At breakfast he couldn't open his yoghurt or his tablet blister pack, asked me to cut his toast and couldn't manage his pyjama buttons. He describes constant burning in his hands, worse at night, and the nurses are helping him with toileting hygiene. He is due to go home tomorrow at 10 am as the main carer for his wife June, who has Parkinson's disease."

B Background

175 words

"Frank fell backwards down two steps at home and is being managed without surgery in a rigid collar for six weeks, so he's halfway through. Physio has documented that he walks with a four-wheeled walker with supervision and manages two steps with a rail. Nursing describe him as pleasant and independent with mobility.

"He lives in a lowset house in Bundaberg with June, who is 75. He says he does 'everything' for her: her time-specific tablets, meals, helping her shower and driving her to appointments. His stepdaughter Kerry has been caring for June since the fall but returns to work Monday and is keen for him to come home. The OT referral only came through yesterday.

"There are important gaps. I don't know whether the nursing or medical team know he's been removing the collar, or whether he has noticed any new symptoms. There's no recorded hand function assessment. I also don't know June's current needs, how her medications have been managed, or whether they have any community services or a My Aged Care assessment."

A Assessment

282 words

"The collar is my first priority. Frank's neck is being treated without surgery, so the collar is what protects his spinal cord while it heals. Taking it off and moving his head to shave and wash could risk further injury to an already damaged cord. The itch also needs checking, because skin under a rigid collar can break down. This needs attention now.

"The second issue is his hands. His presentation fits a central cord pattern, which is typical after a hyperextension injury in an older person. The arms, and especially the hands, are affected

much more than the legs, and burning pain is common. That explains why he walks well but can't manage buttons, packaging or cutting food. Frank says 'the hands will come back', but hand recovery is often slower and less complete than leg recovery, so we can't plan on that.

"Occupationally, Frank currently needs help with dressing, eating, medication access and toileting hygiene. He would also be going home to provide care for June that depends on fine motor skills: opening and giving time-critical Parkinson's medication, helping her shower and cooking. He can't drive while wearing the collar. If he goes home tomorrow as planned, both Frank and June could be at risk, including June missing her medication.

"The discharge plan seems to rely on his mobility. Physio has cleared him to walk with supervision, but walking doesn't tell us whether he can care for himself or for June. Kerry's understandable pressure, the booked transport and the paperwork request don't change that.

"As a student, I can't make medical decisions, assess his capacity or stop a discharge. I do think these findings warrant urgent team review before tomorrow."

R Recommendation

391 words

"Right now, I've asked Frank to sit still, keep his head in a neutral position and not put the collar on himself. I'd call his nurse to refit it according to the ward's procedure, and I'll stay with him until they arrive. I'd like your help to escalate this to the nurse in charge and the spinal medical team today. Medical staff can review him for any new neck pain, weakness or numbness and decide whether anything else is needed. Nursing can check the skin under the collar and set up a safe collar-care routine for shaving and washing. I'd document what happened and complete an incident report if required.

"Second, I'd ask you to support me in raising with the medical team and nurse unit manager that discharge tomorrow may not be safe yet. I'd recommend we hold off on completing the discharge paperwork until his hand function and caring role have been assessed. I'd also suggest medical review of the burning pain, as it may be neuropathic and affects his sleep.

"Third, I'd involve social work urgently to talk with Frank, Kerry and June about June's care, support for Kerry, possible respite, and a My Aged Care referral or transition care if appropriate. Pharmacy could advise on easier-to-open medication packaging for both of them. Physio could advise whether he could safely help June physically.

"For OT, my principal short-term assessment would be observed performance of his morning routine today: dressing, toileting hygiene, managing his medication packs and preparing a simple meal, plus a discussion of the tasks he does for June. I'd trial simple equipment such as a button hook, built-up cutlery and a long-handled sponge, and refer for a home visit if he's going home.

"Longer term, I'd use the COPM with Frank, because caring for June and getting back to lawn bowls clearly matter to him.

"I'd also suggest a family meeting with Frank, June and Kerry before any new discharge date. Frank's role as June's carer is central to who he is, so the plan should support that role, not just remove it.

"The NSQHS Comprehensive Care Standard supports this kind of team-based, person-centred discharge planning. My immediate priority is getting Frank's collar safely back on and the team

> aware, then making sure he isn't discharged tomorrow before we know he and June will be safe."

Coaching notes *not spoken*

The immediate priority

Frank's rigid collar is off despite documented orders to wear it at all times, and he has been removing it daily. Protect his neck now and escalate. The second priority is that his hand function makes tomorrow's discharge as June's carer unsafe without team review.

Distractors in this case

- **Walks with a walker and supervision** — central cord injuries affect the hands more than the legs, so walking ability doesn't predict self-care.
- **"The hands will come back"** — recovery is uncertain; plan for current function.
- **Transport booked and paperwork requested** — logistics shouldn't drive a clinical decision.
- **Kerry's pressure** — understandable, but it is a sign of carer strain to address, not a reason to discharge.
- **Lawn bowls** — a meaningful longer-term goal, not today's focus.

Missing information to clarify

- Whether the team knows about the collar removal; any new neurological symptoms
- Skin condition under the collar
- June's needs, supports and medication management
- Community services, My Aged Care status
- Formal hand function findings

Useful references

- Australian Commission on Safety and Quality in Health Care, *NSQHS Standards – Comprehensive Care Standard*

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